

--

Name: صبرين محمود احمد محمد National ID: 27411070103003	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Trastuzumab Subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٤/٠١

٢٧٤١١٠٧٠١٠٣٠٠٣

رؤية منزل

سكنية

أشرف

المطابقة سارية حتى ٢٠٣١/٠١/٣١



جمهورية دولة فلسطين
سلطة تحقيق الشخصية

صبرين

محمود احمد يوسف

هـ شـ المستوفى عربية خليل

المعمرة - القاهرة

٢٧٤١١٠٧٠١٠٣٠٠٣

KJ8253463

ملاحظة
هذا هو
الرقم
الذي
يستخدم
للمطابقة

Name : Sabreen Mahmoud Ahmed Youssef

Date : 20-09-2025 12:29

Branch : Dokki

Medical No. : 206889

Ref.Physician Prof : Gostaf Roseh Hospital

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 50-year-old lady, with history of breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen all over the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Name: صابرين محمود احمد رياض		
MR. : 1722		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته تاريخ وساعة الدخول : تاريخ وساعة الخروج :		
وبعد الكشف والفحوصات والمتابعة وجد انه PREM METACH BILATERAL BREAST CANCER RT BREAST CANCER 2023 ER8/8 PR8/8 HER2-VE REC NACT AC TAXANES RT MRM YPT2 YPN0/23 2023 REC PORT ZOLADEX NOLVADEX THEN 3/2024 LEFT BREAST CANCER ER3/8 PR-VE HER23+ REC 2 CYCLES CTH THEN REFUSED TO CONTINUE LLEFT MRM 4/2024 Y[T2 YPN2 6/12 REC ONLY 4 PHESGO FEMARA ZOLADEX .CTS 1/2026 D12 LYTIC LESION LEFT FEMORAL LESION .FOR NEW BS AND ASK ABOUT APPROVAL OF HERCEPTIN FOR ZOLADEX FEMARA TODAY patient is not compliant, she received only 3 cycles of herceptin after last MDT on 9/2025,prolia approval not ready yet new bone scan on 04-04-2026: multiple metastatic osseous lesion at left supra orbital, bil scattered ribs, left side LV 5. new approval for herceptin sent, new CT CAP :requested today for femara , due date zoladex next month CT CAP AND HERCEPTIN STILL PENDING APPROVALS today for femara for 3 m + Zoladex (next due date 30/8) next visit 5/7 femara pharmacy the next 2/8 femara pharmacy the next 30/8 femara + zoladex clinic Herceptin approval ???!! for zoladex + femara for 3 months. The patient has completed 2 cycles of CTH.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
	الطبيب المعالج : dina makram	
يعتمد ,		

Name : sabrin mahmoud ahmed youuf

Branch : Portable

Ref.Physician Prof : cap+c

Date : 21-01-2026 02:50

Medical No. : GRI-22080

Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of bilateral MRM, no available previous examination.

I- CHEST:

- Both breasts showed clear operative bed with no evidence of enhanced focal lesions. Associated with mild left subcutaneous edema
- the left upper lobar showed subpleural reticulation with traction bronchiectasis and atelectasis bands with mild right upper lobe subpleural fine reticulations, likely postoperative sequel.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, right suprarenal gland, aorta and IVC. Bulky left suprarenal gland.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Pelvic calcific phleboliths.

Name : sabrin mahmoud ahmed youuf
Branch : Portable
Ref.Physician Prof : cap+c

Date : 21-01-2026 02:50
Medical No. : GRI-22080
Ins.Company :

- The examined skeleton showed D7 lytic lesion and left femoral head sclerotic lesion for further bone scan.

Opinion:

- Bilateral clear operative bed with no enhanced focal lesions.
- Bilateral upper lobar subpleural reticulations being evident on left side likely post-radiation sequel.
- Bulky left sided suprarenal gland.
- D7 lytic lesion and left femoral head sclerotic lesion for further bone scan.

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

Name : Sabreen Mahmoud Ahmed Youssef
Branch : Dokki
Ref.Physician Prof : Goustav Rousey Egypt

Date : 04-04-2026 09:42
Medical No. : 341850
تعاقب مستشفى : Ins.Company

Tc-99m MDP SKELETAL SCINTIGRAPHY

- **CLINICAL HISTORY:** 50-year-old female patient; history of breast cancer, complains of bony pain; referred for metastatic survey.
- **Technique:** Anterior and posterior whole-body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Few randomly distributed osseous lesions "in keeping with metastatic osseous lesions" are seen at left supra-orbital region, bilateral scattered ribs, and left side of LV5.
- Osteoarthritic features are noted around big joints

IMPRESSION:

- Few active osseous lesions as described, likely metastatic, for radiological correlation and follow-up.

Much oblige

Dr. Alaa Nagi, MD
Nuclear medicine

Patient's Name:

Sabreen A

Date:

2026-06-11

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Known case of bilateral breast cancer
- Left upper lobar subpleural reticulations and fibrotic strands likely post therapeutic sequel
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatic lesions.
- ❖ Left upper lobar subpleural reticulations and fibrotic strands likely post therapeutic sequel

Name : Sabreen Mahmoud Ahmed Youssef

Date : 04-04-2026 09:42

Branch : Dokki

Medical No. : 341850

Ref.Physician Prof : Goustav Rousey Egypt

تعاقب مستشفى : Ins.Company

Tc-99m MDP SKELETAL SCINTIGRAPHY

- **CLINICAL HISTORY:** 50-year-old female patient; history of breast cancer, complains of bony pain; referred for metastatic survey.
- **Technique:** Anterior and posterior whole-body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Few randomly distributed osseous lesions "in keeping with metastatic osseous lesions" are seen at left supra-orbital region, bilateral scattered ribs, and left side of LV5.
- Osteoarthritic features are noted around big joints

IMPRESSION:

- Few active osseous lesions as described, likely metastatic, for radiological correlation and follow-up.

Much oblige

Dr. Alaa Nagi, MD
Nuclear medicine

Patient's Name:

Sabreen A

id Youssef

Date:

2026-06-11

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISCICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- o Known case of bilateral breast cancer
- o Left upper lobar subpleural reticulations and fibrotic strands likely post therapeutic sequel
- o No pulmonary nodules, masses or cavitary lesions.
- o No consolidation or collapse.
- o No hilar or mediastinal masses or lymph node enlargement.
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatic lesions.
- ❖ Left upper lobar subpleural reticulations and fibrotic strands likely post therapeutic sequel